

PLAZILIN

METOCLOPRAMIDE INJECTION BP 5MG/ML, 2 ML AMPOULE

Description and Composition:

Description: Clear, almost colorless solution.

Composition: Each mL contains Metoclopramide Hydrochloride BP 5mg

Pharmacodynamics

Gastrokinetic Effects

Metoclopramide restore the normal motility of upper G.I tract without affecting gastric, biliary or pancreatic secretions. Action of metoclopramide on the motility of stomach are unaffected by vagotomy or pyloroplasty. Metoclopramide increase the amplitude of lower esophageal contractions and lower esophageal sphincter pressure (LESP) and thus prevents gastroesophageal reflux (GER).

Metoclopramide increase frequency and depth of antral contractions, restores pyloric sphincter competence, regularises gastric pyloric, duodenal and jejunal motility, thereby resulting in accelerated gastric emptying and intestinal transit, thus preventing duodenogastric reflux. It has little, if any, effect on the motility of colon or gall bladder.

Antiemetic effects

Antiemetic effects of metoclopramide result from its central and peripheral actions. Apart from this, metoclopramide decrease the sensitivity of visceral nerves which transmit afferent impulse from G.I tract to the emetic centre in the lateral reticular formation.

Pharmacokinetics

Metoclopramide is weakly bound to serum proteins (30%) and is rapidly and widely distributed in most tissues (Vd is 3.5 L/kg). Metoclopramide is eliminated by both hepatic metabolism (principally N-4 sulphate conjugation) and renal excretion. About 80% of an dose is recovered in the urine within 24 hours either as the unchanged drug (20%) or its sulphate and glucuronide conjugates. The half-life is 3 to 5 hours in patients with normal renal functions, but it is prolonged to about 14 hours in patients with severe renal failure, necessitating a reduction in dosage. Onset of pharmacological action of metoclopramide is as follows:- 10 to 15 minutes following an intramuscular administration. - 1 to 3 minutes following an intravenous administration.

Indications

Adult population

PLAZILIN is indicated for use in adults for:

- Prevention of post-operative nausea and vomiting,
- Symptomatic treatment of nausea and vomiting, including nausea and vomiting induced by migraine attacks,
- Prevention of radiotherapy induced nausea and vomiting.

Pediatric population

PLAZILIN is indicated in children aged 1 to 18 years for:

- Prevention of delayed Chemotherapy induced nausea and vomiting as a second-line option,
- Prevention of postoperative nausea and vomiting as a second-line option.

Recommended dose and administration

The solution can be administered by the intravenous or intramuscular route.

The intravenous doses must be administered as a slow bolus (for at least 3 minutes).

All indications (adults)

A single 10 mg dose is recommended for the prevention of postoperative nausea and vomiting.

The recommended dose for the symptomatic treatment of nausea and vomiting, including nausea and vomiting induced by migraine attacks and for the prevention of radiotherapy induced nausea and vomiting is 10 mg per dose, 1 to 3 times daily. The maximum recommended daily dose is 30 mg or 0.5 mg/kg.

Treatment duration when administering by injection should be as short as possible and a switch to administration via oral or rectal route should be instituted as quickly as possible.

All indications (children from 1 to 18 years of age)

The recommended dosage is 0.1 to 0.15 mg/kg, 1 to 3 times daily, by intravenous route.

The maximum daily dose is 0.5 mg/kg.

Age	Body Weight	Dose	Frequency
1-3 years	10-14kg	1mg	Up to 3 times daily
3-5 years	15-19kg	2mg	Up to 3 times daily
5-9 years	20-29kg	2.5mg	Up to 3 times daily
9-18 years	30-60kg	5mg	Up to 3 times daily
15-18 years	Over 60kg	10mg	Up to 3 times daily

For the prevention of delayed chemotherapy-induced nausea and vomiting, the maximum treatment duration is 5 days.

For the prevention of postoperative nausea and vomiting, the maximum treatment duration is 48 hours.

Frequency of administration:

A minimum interval of 6 hours between two administrations is to be respected, even if vomiting or rejection of the dose occurs.

Special populations

Elderly subjects

In elderly subjects, a dose reduction should be considered, based on kidney and liver function, and overall frailty.

Kidney failure

In patients with end-stage kidney failure (creatinine clearances 15 ml/min), the daily dose should be reduced by 75%.

In patients with moderate to severe kidney failure (creatinine clearance between 15 and 60ml/min), the dose should be reduced by 50%.

Liver failure

In patients with severe liver failure, the dose should be reduced by 50%.

Other pharmaceutical forms may be more suitable for these patient populations.

Pediatric population

Metoclopramide is contraindicated in children aged less than one year.

Mode of administration

Slow Intravenous or Intramuscular route.

Contraindication

- Hypersensitivity to the active substance or to any of the excipients listed
- Gastrointestinal haemorrhage, mechanical obstruction or gastrointestinal perforation for which the stimulation of gastrointestinal motility constitutes a risk
- Confirmed or suspected pheochromocytoma, due to the risk of severe hypertension episodes
- History of neuroleptic or metoclopramide-induced tardive dyskinesia
- Epilepsy (increased crises frequency and intensity)
- Parkinson's disease
- Combination with levodopa or dopaminergic agonists
- Known history of methaemoglobinaemia with metoclopramide or of NADH cytochrome-b5 deficiency
- Use in children less than 1 year of age

Warning and precautions

On no account should PAZILIN injectable be diluted for injection as this will upset the isotonicity and stability of the drug. IV injection of metoclopramide should be given slowly over 1 to 2 minutes because rapid administration may cause intense feeling of anxiety and restlessness followed by drowsiness. The recommended dosage of Metoclopramide need not be exceeded, as increasing the dose does not produce a corresponding increase in clinical response. WARNING -Avoid doses exceeding 0.5mg/kg/day. - Extrapyra-

midal effects, especially dystonic reaction of metoclopramide are more likely to occur in children shortly after initiation of therapy, and usually with doses higher than 0.5mg per kg of body weight per day.

Special Warnings and Precautions for Use

Neurological disorders

Extrapyramidal disorders may occur, particularly in children and young adults, and/or when high doses are used. These reactions generally occur at the beginning of treatment, and can occur after a single dose. If extrapyramidal symptoms occur, metoclopramide should be discontinued immediately. These effects are generally completely reversible after treatment discontinuation; however, symptomatic treatment may be required (benzodiazepines in children, and/or anticholinergic antiparkinsonian medicinal products in adults). An interval of at least six hours should be respected between each dose even if vomiting or rejection of the dose occurs, in order to avoid overdose.

Long-term treatment with metoclopramide may cause potentially irreversible tardive dyskinesia, particularly in elderly subjects. Treatment should not exceed 3 months because of the risk of tardive dyskinesia. Treatment must be discontinued if clinical signs of tardive dyskinesia occur.

Neuroleptic malignant syndrome has been described with metoclopramide in combination with neuroleptics and with metoclopramide monotherapy. Metoclopramide must be immediately discontinued if symptoms of neuroleptic malignant syndrome develop, and appropriate treatment should be initiated.

Particular caution should be exercised in patients with underlying neurological disorders, and in patients receiving other centrally acting drugs.

Symptoms of Parkinson's disease may also be exacerbated by metoclopramide.

Methemoglobinemia

Methemoglobinemia, which could be related to NADH-cytochrome b5 reductase deficiency, has been reported. If this occurs, treatment must be immediately and permanently discontinued, and appropriate measures initiated (such as treatment with methylene blue).

Cardiac disorders

Serious cardiovascular undesirable effects, including cases of severe bradycardia, circulatory collapse, cardiac arrest and QT prolongation have been reported during administration of metoclopramide by injection, particularly via the intravenous route.

Particular caution should be exercised when administering metoclopramide, particularly via the intravenous route, in elderly subjects, patients with cardiac conduction disorders (including QT prolongation), patients with electrolyte imbalance, bradycardia, and patients taking other drugs known to prolong QT interval.

The intravenous injection must be given as a slow bolus (of at least 3 minutes duration) in order to reduce the risk of undesirable effects (e.g. hypotension, akathisia).

Kidney or liver failure

In patients with kidney failure or severe liver failure, a dose reduction is recommended.

Pregnancy and Lactation

Animal tests have shown no teratogenic effect with metoclopramide. Although there had been no reported teratogenic effect following the use of metoclopramide during pregnancy, there are no adequate and well controlled studies in pregnant women. Hence, metoclopramide should be avoided in the first trimester of pregnancy and could be used during second and third trimester of pregnancy only if clearly indicated. Metoclopramide is excreted in breast milk and hence caution should be taken when it is administered to a nursing mother.

Interactions with other medicaments

The effects of Metoclopramide on gastrointestinal motility are antagonized by anticholinergic drugs and narcotic analgesics. Additive sedative effects can occur when Metoclopramide is given with Alcohol, Sedatives, Hypnotics, Narcotics or Tranquilizers. The finding that Metoclopramide releases catecholamines in patients with essential hypertension suggests that it should be used cautiously, if at all, in patients receiving monoamine oxidase inhibitors.

Absorption of drugs from the stomach may be diminished (e.g., Digoxin) by Metoclopramide, whereas the rate and/or extent of absorption of drugs from the small bowel may be increased (e.g., Acetaminophen, Tetracycline, Levodopa, Ethanol, Cyclosporine).

Gastroparesis (gastric stasis) may be responsible for poor diabetic control in some patients. Exogenously administered insulin may begin to act before food has left the stomach and lead to hypoglycemia. Because the action of Metoclopramide will influence the delivery of food to the intestines and thus the rate of absorption, insulin dose or timing of dosage may require adjustment.

Side effects

Metoclopramide is generally well tolerated and may rarely produce few adverse drug reactions when given in therapeutic doses. ADRs are usually mild, transient and reversible on withdrawal of the drug. ADRs like restlessness, drowsiness, fatigue, lassitude, extrapyramidal effects (1 in 500, readily reversible), tardive dyskinesia, Parkinsonian like symptoms, motor restlessness (akathisia), galactorrhoea and gynaecomastia may rarely occur. Anti Parkinsonian drugs of the anticholinergic type may be used to counteract these infrequently seen reactions.

Symptoms and Treatment of over dose

Metoclopramide in therapeutic doses is well tolerated. Overdoses and antipsychotic doses upto 100 times the recommended dose of metoclopramide have been tolerated without serious side effects.

Metoclopramide does not affect blood pressure, pulse rate or ECG - the three parameters of cardiac function.

Storage condition

Store below 30°C. Protect from light.

Shelf Life

3 years

Incompatibilities

If this product is used for the treatment of nausea and vomiting associated with cytotoxic drugs, the cytotoxics should be administered as a separate infusion.

Dosage Forms and packaging available

1 ampoule of METOCLOPRAMIDE INJECTION BP 5MG/ML, 2mL per ampoule. 10 ampoules are arranged in a tray and packed in a secondary carton along with the package insert.

Manufactured by

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Product Registration Holder/Imported by:

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